

T H E

Pregnancy Protocol

N O U R I S H E V E R Y T R I M E S T E R .

B Y

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GUTGURU PROTOCOL

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CHAPTER 01

Nutrition Across *All* Trimesters

Each trimester has distinct nutritional priorities. Getting this right reduces risk of complications and supports optimal foetal development.



First Trimester (Weeks 1–12)

Neural tube formation. Folate is critical. Food aversions are normal — focus on what you can tolerate. Nausea is usually B6-responsive.



Second Trimester (Weeks 13–26)

Rapid foetal growth begins. Protein and iron needs increase sharply. Blood volume rises — iron deficiency anaemia becomes the key risk.



Third Trimester (Weeks 27–40)

Brain development is most active. DHA, choline, and calcium are the priority nutrients. Calorie needs are at their highest (+450 kcal/day).

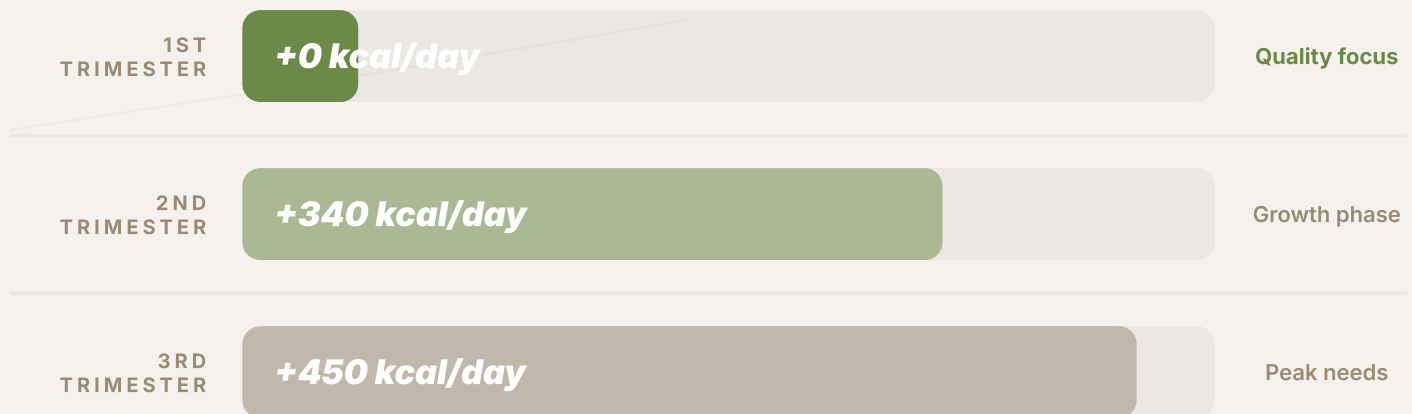


Throughout Pregnancy

Hydration (2.5L/day), fibre (25–35g), and small frequent meals stabilize blood sugar, prevent constipation, and reduce gestational diabetes risk.

CHAPTER 02

Calorie Needs *by Trimester*



The first trimester requires no additional calories — only nutritional quality. The most common mistake is overeating in T1 and under-eating in T3, when calorie needs are actually highest.

KEY TAKEAWAY



Protein is the most important macronutrient in pregnancy. It builds every cell of the developing baby. Target 1.1–1.3g per kg body weight per day — more than standard adult guidelines.

CHAPTER 03

Safe vs Avoid in Pregnancy

Food safety is as important as nutrition during pregnancy. Some risks accumulate gradually — consistency matters.

SAFE & BENEFICIAL

Pregnancy Superfoods

These deliver the nutrients most critical across all three trimesters.

Palak / methi Ragi Rajma / chana
Fully cooked eggs Milk + curd Dates
Walnuts Moringa Rohu / pomfret (cooked)
Coconut water Brown rice

AVOID

Avoid in Pregnancy

These carry infection, hormonal, or developmental risks to the foetus.

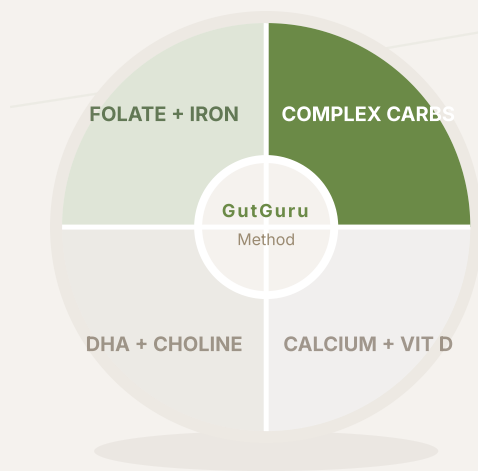
Raw papaya Alcohol (zero tolerance)
Raw / undercooked meat Unpasteurised dairy
High-mercury fish Raw sprouts
Excess licorice
Large amounts of vitamin A supplements
Excess caffeine

Zero alcohol is the only safe threshold in pregnancy. No "safe" amount has been established by any clinical body — the risk is dose-dependent and cumulative.

CHAPTER 04

The Pregnancy *Plate* Formula

THE GUTGURU ALL-TRIMESTER METHOD

**Complex Carbohydrates**

Ragi, dalia, brown rice, rajma, oats

+

**Folate + Iron Rich**

Palak, methi, moringa, rajma, dates

+

**DHA + Choline**

Eggs, walnuts, flaxseed, rohu (2×/wk)

+

**Calcium + Vitamin D**

Milk, curd, ragi, sesame, paneer, sun

EXAMPLE MEAL

Ragi roti + rajma + palak + curd + 2 walnuts + egg**T1: +0 kcal · T2: +340 kcal · T3: +450 kcal above
baseline**

CALORIE ADDITION BY TRIMESTER

CHAPTER 05

Common *Pregnancy* Mistakes

**"Eating for two" from the very first week**

Additional calories are needed from the second trimester — not from week 1. Over-eating in T1 increases gestational weight gain without benefiting the baby.

**Eating raw or undercooked eggs, meat, or shellfish**

Listeria and Salmonella carry significant risk during pregnancy — including preterm labour and foetal infection. All animal proteins must be fully cooked.

**Not drinking enough water throughout the day**

Dehydration in pregnancy is associated with Braxton Hicks contractions, urinary tract infections, and reduced amniotic fluid levels.

**Avoiding exercise entirely out of fear**

Moderate exercise (walking, swimming, prenatal yoga) is safe and beneficial in uncomplicated pregnancies — improving outcomes for both mother and baby.

**Ignoring protein in the second and third trimesters**

Protein builds every cell, tissue, and organ of the developing baby. The requirement increases to 1.1–1.3g/kg/day in the second half of pregnancy.

CHAPTER 06

Quick Myth Busters

MYTH

Eating ghee in the last trimester makes delivery easier.

TRUTH

There is no clinical evidence for this. Excess ghee in T3 adds unnecessary saturated fat and calories. Use ghee in moderation as a cooking fat throughout pregnancy.

MYTH

Raw papaya is the only fruit to avoid in pregnancy.

TRUTH

Raw papaya and large amounts of pineapple (bromelain) carry uterine stimulation risk. All other fruits are safe and encouraged as part of a varied prenatal diet.

MYTH

If you feel very sick in the first trimester, the baby isn't getting nutrients.

TRUTH

Nausea is driven by rising hCG — a sign of a healthy pregnancy. The foetus has nutritional priority in early pregnancy. Focus on tolerating what you can.

TAKE THIS HOME

Your 3 Quick Wins

Three specific actions you can start this week — no equipment, no cost, measurable results.

1

Add Palak or Methi to One Meal Every Day

Add palak or methi to at least one meal daily — in dal, paratha, sabzi, or soup. These two leafy greens deliver the folate and iron most needed across all three trimesters. Simple, affordable, and clinically impactful.

FROM TODAY UNTIL DELIVERY

2

Drink 8–10 Glasses of Water Spread Across the Day

Dehydration in pregnancy causes Braxton Hicks contractions and increases UTI risk. Drink a glass first thing in the morning, one with each meal, and one between meals. Set a phone reminder at 10am, 1pm, 4pm, and 7pm if you forget.

DAILY — SET A REMINDER IF NEEDED

3

Eat 2 Walnuts + 1 Tsp Flaxseed Daily as Your DHA Snack

2 walnuts and 1 teaspoon of ground flaxseed is your daily DHA action — plant-based, affordable, and takes 10 seconds. This delivers ALA omega-3 for foetal brain development when fatty fish is not available or tolerated.

ONCE DAILY, EVERY DAY

MORE AT www.thegutguru.co.in →



READY TO PERSONALIZE?

Book a 1-on-1 Consultation

Get a clinical nutrition plan tailored to your pregnancy nutrition
across all trimesters, body, and lifestyle.

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